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Original Article

Building self-advocacy in HIV care: the use of role-play to examine healthcare access for HIV-positive MSM in rural South Africa

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Abstract: There is high HIV prevalence and low rates of viral suppression for men who have sex with men (MSM) in South Africa, with few MSM-centered interventions to address these outcomes along the HIV treatment cascade. Participatory interventions may support community building among HIV-positive MSM through which they can share approaches of self-advocacy that are contextually grounded. We conducted a pilot study to assess the use of role-plays in influencing social isolation while also updating our understanding of MSM healthcare experiences in Mpumalanga, South Africa. The study was conducted with 21 MSM leaders who were HIV-positive. There were three groups of seven participants each who created and performed role-plays based on their healthcare experiences, with a focus group discussion (FGD) conducted afterward. Audio-recordings were transcribed, translated, and analyzed using a constant comparison approach. We found that MSM described role-play as cathartic and a future HIV care educational tool for other MSM, and that they outlined points of self-advocacy during HIV care in clinics. Our study suggests that future research should utilize role-play so to integrate contextual factors influencing HIV treatment, especially in high HIV prevalence settings.

Keywords: Men who have sex with men (MSM), HIV care, role-play, South Africa, coming out

Introduction

Barriers to HIV care for men who have sex with men (MSM) include social isolation and a history of unwelcoming clinical environments due to sexuality stigma in sub-Saharan African countries (1–3). As a result, MSM are often lost to follow-up after testing HIV-positive (4,5). Since most MSM live with family, many find it difficult to disclose their HIV status to them once they are diagnosed because they fear that this will lead to a conversation about their sexuality, which may lead them to experience abuse, being kicked out of the home, and managing their HIV disease alone without support (6,7). Further, clinics have a history of perpetuating stigma and discrimination based on sexuality (8,9). Although there has been significant MSM-health competency training provided to healthcare providers, like nurses and doctors, in

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select African clinical settings such as in Kenya and South Africa, healthcare workers remain poorly equipped to counter the household barriers that create social isolation and prevent MSM from returning to care (9–11). Consequently, MSM will not return to clinic until after they develop advanced HIV disease and will not disclose their HIV status to close friends due to HIV gossip in the MSM community (12,13). Although recent research shows that once in care, a significant proportion of African MSM are on antiretroviral therapy (ART), only an estimated 25% are virally suppressed across African countries where there is 30.4% HIV prevalence; and in South Africa, HIV prevalence range is as high as 48.2%, with only an estimated 20% virally suppressed (1,2). Thus, it is imperative that we examine how social isolation can be addressed while also providing an updated perspective of the clinical experience for MSM in order to improve their earlier linkage to care. Role-play may offer a tool to both examine HIV care access and provide support for HIV-positive MSM in African settings.

Theatre-based HIV research

Theatre or popular theatre are common names used for a community-based participatory research method that is designed to work with individuals to develop a play based on their shared health experiences, and then perform this play for others who experience this health condition in order to identify and define shared experiences and build community (14–17). This method has been utilized to engage with communities in HIV prevention and treatment to include education that may mitigate stigma globally with extensive use of theatre-based approaches in African settings since the beginning of the response to the HIV epidemic (15,18–20). Theatre has been shown to engage and bridge language and literacy to convey complex concepts like the microbiology of HIV to common experiences such as negotiating safe sex with partners, if these are tailored to context and people involved. Developing and participating in theatre performances about HIV in front of others has demonstrated retention of skills learned and empowerment, such as for university students and those living with HIV in Malawi (20,21). Skits and theatre focused on HIV prevention and treatment have been shown to be highly acceptable and viewed within African communities, leading to increased HIV testing and counseling, and acceptability of HIV disclosure within families (22,23). In several projects employing role-play and theatre in both African and Western settings, it was found that group-developed performances created a platform for participants to express their feelings about HIV without any limitations or filter (14,16,18). As such, they were able to feel a sense of solidarity and comfort with each other, building social capital (14,16,20,24). Specifically, the use of role-play allowed them to clarify their personal views of sexual health and identify perspectives that either aligned or did not align with others; a process through which participants could then learn more about themselves (25). Among the sharing of experiences, participants could feel a greater sense of acceptance of themselves and past experiences and find comfort in talking about their sexual health and reducing a sense of isolation (14,15,23,24).

Few theatre-based HIV research studies have been designed for MSM in African settings. Therefore, we worked HIV-positive MSM to develop HIV-related plays in Mpumalanga, South Africa. The objective was to understand how plays developed connectedness between participants and a shared examination of their health and clinical care access in order to inform future research in this area.

Methods

We used theatre-based methods to develop 'role-plays' that examined HIV and healthcare experiences for HIV-positive MSM attending public clinics in Mpumalanga. This study was part of a larger, long-term mixed methods study (2013–2018) conducted in the same setting (3,4).

Participant recruitment

The study was conducted in two rural districts of Mpumalanga Province, a setting where one in two MSM over the age of 25 years is HIV-positive (2). We recruited 21 HIV-positive MSM leaders living in three different rural township and villages in Mpumalanga. We utilized purposive participative sampling of an already established cohort of HIV-positive MSM, and recruited MSM who were 18–25 years of age and leaders within their local social network (26). In this setting, MSM leaders are those who are the central figure in organizing social activities for a group of other MSM. Leaders were

identified based on their engagement with the other research activities (photovoice, interviews, and focus groups) and their discussion of their leadership role in this larger study, with additional details of the cohort recruitment outlined elsewhere (3,4). The recruitment resulted in three groups of MSM with seven participants per group who completed this phase of the study, with one group conducting two role-plays because two townships were represented. Each group attended one six-hour session on average per role-play, with snacks and lunch provided.

Role-play design

Participants wrote and performed short role-plays based upon their shared HIV healthcare experiences (14,25). The role-play development process involved three basic steps: individual clinical stories, play writing, and rehearsal.

Individual clinical stories

Each member of the group was asked to reflect on their healthcare experiences for their HIV treatment, including factors influencing their health, and think of an experience that signifies this reflection and one that they would like to share with the group. Afterward, each member shared their experience as the other group members listened. One group member was tasked to take basic notes about each story.

Play writing

Once all members of the group shared their experiences, the group was tasked with writing a role-play that incorporated at least one element from each story. Groups were provided poster paper to write down ideas and the formal script, with character names with backstories and initial play staging. Participants were encouraged to use or create props from the room to enhance their play. Role-plays were conducted in townships in a carport, and a shack, and one set of plays were conducted in a health program space in a central business district.

Rehearsal

Each group rehearsed their play with staging. This rehearsal involved all participants playing a role, and it was emphasized that the character being played does not necessary reflect the perspective of the actor; meaning that an actor playing the role of someone who demonstrates homophobic behaviors does not imply that the actor is homophobic.

Performance

After each group felt prepared, then each group performed their play in their preferred language (English, IsiZulu, and IsiSwati), which was audio-recorded and videotaped for the group to watch again afterward to support the FGD. Also, the videotaping was designed to create a performance atmosphere for the plays. After participants performed their role-play, we conducted a semi-structured FGD to discuss the play(s) and how these align with their personal experiences with HIV health and clinical care (27). The FGDs were guided by three general domains: (a) how the role-play was and was not a shared experience; (b) how it captured healthcare decision-making and why; and (c) what are the potential next steps in addressing the health issue of concern in the role-play.

Data analysis

We conducted the role-plays and FGDs in their preferred language (English, IsiZulu, and IsiSwati) with code-switching. The audio-recordings of the role-plays and FGDs were transcribed and translated into English for analysis. Video-recordings were used to support translation of the play into English, but no visual analysis was conducted with the videos for this study. Also, the written play scripts created by the participants were used to triangulate with audio-recordings of the plays to confirm transcription accuracy.

The analysis involved a series of meetings conducted over six months. During the first three meetings, the researchers reviewed the translated role-plays and created a synopsis for each. This process involved research writing these separately and then discussing each to confirm the content of the play. After confirming the play storylines, the research team focused on analyzing the FGD data. We analyzed the final FGD transcripts utilizing a constant comparison approach where codes and themes were developed in an inductive process (12,28). Our approach included weekly meetings discussing these codes to include the creation of memos in order to generate associations

between codes and thematic development. We also incorporated researcher observation notes about the research process to ensure that everyone understood cultural terms and the context about HIV and sexuality that would help us answer our research questions (27). Two researchers collected the data and then coded the transcripts separately first and then met to discuss their coding to identify themes that served as findings from the study. After corroborating the findings, we identified archetype quotes from play discussions to represent about HIV and healthcare experiences of this MSM community.

Ethics

We received human subject approval for the study by institutional review boards at the University of California Los Angeles in the US and the University of the Witwatersrand in South Africa. All participants are represented by an alias and initial of the township (WT, CC, EL) where the role-play was created in order to maintain confidentiality. Finally, participants used the term 'MSM' frequently to identify themselves, and so we used this term with occasional uses of 'gay' as appropriate.

Results

There were four role-plays developed by MSM leaders in four township settings in Mpumalanga, South Africa (Appendix 1). The role-plays examined MSM experiences in public clinics and traditional care, coming out to family, and completing the theatre method itself. We outline why the participants presented these stories next.

Sharing experience, gaining knowledge

Participants found the role-play method cathartic because they were able to discuss their perspectives about coming out and experiences in healthcare with other MSM for the first time. Most of the participants had not been in a situation where they could openly discuss being HIV-positive with others in their community and also learn that they shared common experiences. As one participant stated, 'I think we should do this more often. You have woken up a part of our souls. I didn't know I could act!' (Mxolisi in WT). Participants were surprised that they could create a play about their experiences and then act it out as a way to share that experience with others in their community. This process had tremendous value to them as they enjoyed meeting with and learning from other HIV-positive MSM.

Furthermore, MSM found the role-play method freeing and empowering as they had performed their unique role-plays in front of an audience, and they discussed about how role-plays can serve as an educational tool for other HIV-positive MSM who need to access care:

> To me, it just shows the things [that] we [all] experience at the clinic so we can see [understand] exactly what they [nurses] do [that makes us uncomfortable or supported]. [Lerato in WT]

> And you know the steps to take when they are not treating you well.... You're getting more knowledge [by watching the plays]. Learning. Because people out there may talk [other MSM who are HIV-positive and don't go to clinic], but they don't really know what goes on at the clinic. So, if you [were one of these MSM and] watched these plays, you can see that they [clinic staff] don't always mistreat us. Sometimes they treat us well. [Nathi in WT]

Here, in a discussion between two participants, they described that there is a misinterpretation among those in the MSM community about what is really happening in healthcare clinics and amongst staff there. MSM described that not all clinical staff treat MSM uniformly negatively, that there are some who treat them well, and this supportive treatment should not be disregarded or ignored. They saw role-play as an educational tool for other MSM for them to understand this misinterpretation, but also acknowledged that stigma and discrimination remain an issue, and role-play can provide tools for MSM to navigate their healthcare experience because they must attend clinic for their HIV treatment in order to say healthy.

Trying to gain autonomy and control in their clinical care

Most participants discussed their desires or acts of independence in their clinical care. As all participants were 25 years of age and living at home, they often relied on their parents to take them to the clinic.

While they depend on family, participants wanted independence in their HIV care. Yet, their feeling of autonomy is tempered by financial dependence, the support that they need, and how they are perceived as young adults.

> It's because people don't take the youth seriously these days. [Samgelo in EL]
>
> Do you go to the doctor with your parents? [Researcher]
>
> No, we are adults now. [Mxolisi in EL]
>
> So, would you ever go to the doctor by yourself? [Researcher]
>
> Yep, so many times. [Samgelo in EL]
>
> Sometimes because we need them [family] to comfort us when the situations are different [when they are really sick]. [Also], sometimes she's [mother] the one who'll pay for everything because she got medical aid. [Mandla in WT]
>
> When I go to the special doctor, I need my mother to pay for it. That's why I need my mother [and] other times [I need her too because] I don't have strength and I need someone to sign [forms for me], then yeah. [Prince in WT]

While MSM may want to manage their healthcare themselves, we found most contend with not being taken seriously as young adults in the clinic and by the basic fact that some MSM need their family. Many MSM explained that they are unable to be completely autonomous because they are still financially dependent on their parents, especially with regard to health insurance and bill payments. Also, they rely on and desire their parents' support when they are really sick and need to go to the clinic, during which time their parents help with the administrative matters like signing hospital forms.

Yet, parental support of HIV care is also linked to MSM coming out as gay or bisexual to them, which may include extended family members, and is reflected in the *Coming Out* play.

> We wanted to show that being gay starts at an early age. Our friend, Bafana, started liking boys at an early age. He hadn't had sex yet, but he knew that he liked boys. We also wanted to send the message that parents must take care of their children and accept them for who they are. . .. So, Sam [Bafana's uncle] gives him some advice. . . that he should take of himself [practice safe sex] because he knows that people love sex and he has experience with gay people. [Sebastian in EL]

In *Coming Out*, Bafana tells his uncle that he is gay because he wants his uncle to help him come out to his parents so that they will support his life and health. Often, uncles or aunts are intermediaries or the first family to whom someone discloses a social or health emergency, such as Thabiso who is taken to a Sangoma, a traditional healer, for treatment for his crabs by his uncle (Muti is not for Crabs, EL). During Bafana's coming out process, his uncle, Sam, tells him to practice safe sex to protect him, given the HIV risk in the community, but his uncle does not know that Bafana is already HIV-positive. For many MSM, it is difficult to come out to parents as HIV-positive without also coming out as gay or bisexual first because it is perceived that parents will ask how they got HIV if they don't have a girlfriend. At the end of the play, Bafana does come out to his parents, who are supportive of his identity, but participants admit that such immediate support is uncommon. Yet, while participants desired parental support for their identity and HIV care, they also pursued independence by going to the clinic on their own some of the time, which resulted in uncomfortable interactions with other patients and healthcare staff.

MSM noticed that they were treated differently from other patients when they went to clinic alone without their parents. While in the waiting room, many participants described a sense of being judged or experienced being discriminated against by other patients.

> What are the big main points that they are trying to get across in the story (Waiting Room Trials)? [Researcher]
>
> Discrimination [by other patients in the waiting room]. [Franklin in WT]

> Yes. Because if you look at the story, some people think that because you choose to be gay, you are somehow afraid of girls. [Sihle in WT]

Many participants described a sense of anxiety while in the waiting room. Not only were they navigating HIV stigma but also stigma based on their sexuality as in the comment above. Participants described a sense of compounded vulnerability as one stated: 'Because of [our] HIV status, [we] are labeled and gossip ensues [about our sexuality]. Once they have been labeled [us], receiving treatment is extremely difficult. . .[we] are gossiped about by patrons waiting to see the doctor' [Sibusiso in CC]. MSM find waiting rooms a difficult place to stay, describing a constant practice of defending themselves to other patients, as in the play, *Waiting Room Trials*. Participants described an increased curiosity from nurses and doctors about their lives and felt the need to have to over explain their sexuality, their relationships, their talk, and dress. They believed that the additional probing questions had nothing to do with receiving treatment, and they see that other patients who are in the waiting room are not subjected to the same amount of attention. When asked if nurses are just being curious, one participant stated:

> I think so because sometimes they [nurses] ask these funny questions that they don't ask straight people. [As they ask these questions I say to myself], 'Oh, I know that I am gay at an early stage, and me being gay doesn't involve any of you [nurses]'. But, I do say, 'Actually, I am not here for that [to discuss being gay]'. [Lunga in WT]

Most MSM didn't describe experiencing discrimination every time that they visited a clinic, but they did experience additional lines of inquiry from nurses about their sexuality that was more curious than judgmental, yet still these questions made MSM feel uncomfortable and most advocated for themselves by asking the nurse to stop asking such questions.

Discussion

There are high HIV prevalence and low viral suppression rates due to limited healthcare access for MSM in South Africa (1,13). There is an urgent need to improve these outcomes, and theatre-based research is a potential method that may be used to examine healthcare access barriers in order to develop responsive community-based interventions (4,11,15,17,19,29). This study was conducted to assess the acceptability of the role-play method for HIV-positive MSM including its ability to develop community and to understand their current healthcare access in Mpumalanga, South Africa (14,16,19). We found that participants described the method as cathartic, with plays created and performed about self-advocacy in household and healthcare settings that were subsequently elaborated upon during FGDs.

Role-play was cathartic, generating shared clinical experiences

We found that MSM experience significant barriers to their HIV care in the household in that they are reluctant to come out as gay or bisexual to their family because they fear that they will not be supported or will be kicked out of the house (7). Coming out is linked to HIV disclosure because MSM find it difficult to talk about their newly diagnosed HIV status to family with whom they live without also explaining how they acquired HIV, because their family ask this question (4). Thus, MSM will not tell their family their HIV status and sexuality, nor will they tell their close friends about their HIV status because they fear gossip (3). As in other settings, MSM are often lost to follow-up in a clinic for up to two years, during which time they are socially isolated from family and friends with regard to their HIV health, until they become ill with advanced HIV disease, when they do have family take them to the clinic and initiate treatment (8). There is a vacuum of social support for HIV-positive MSM, but we found that role-play offered participants an experience to tell their HIV care stories together, creating a play to perform in a group setting that most had never experienced before (18,20,24,25). Through this process, participants learned how others navigated coming out and clinics, including understanding that they had shared stresses, anxieties, and desires around their sexual identify and HIV status. Furthermore, they learned about how others advocated for themselves in the waiting or consultation rooms, or at least how others

desired to self-advocate. Ultimately, role-play was seen as a method to teach other MSM in the community about the importance of coming out and HIV care, as well as how to navigate negative attitudes from other patients and to manage conversations with nurses so to remain on treatment.

The message of self-advocacy in health

Across all the role-plays, MSM conveyed a consistent message that they had a right to healthcare as demonstrated by the South African constitution, and they created role-plays that showed how this right might be challenged in subtle ways, and then how to respond to these challenges (15,30). From the story about responding to other patients in the waiting room, to a nurse who openly judged and devalued gay men during HIV testing, MSM described how to respond directly with their voice and through clinic administrative systems. Through discussions about the role-plays, we found that MSM described both negative and positive clinical experiences with nurses and doctors. In many of these instances that were described, we observed that some health professionals were presented as supporters, and MSM were educating them about their lives with the objective of improving clinical care such as reducing the number of uncomfortable questions about their sexuality. Although sexual and gender minority health competency training for healthcare providers has had some positive impact in some African settings, including South Africa, it is important to note that many still experience blatant discrimination and stigma that hinders their healthcare access and treatment (11,31). Despite the positive benefits of this training, in our study, MSM were not fully comfortable with the increased clinical attention on their sexuality, which some addressed by taking a direct approach in ending this line of inquiry from healthcare providers with phrases like, 'I'm not here for that'.

Ultimately, we found that HIV care for MSM in public clinics remains nascent, with increased need to expand health professional training to include families to mitigate the negative effects of social isolation on linkage to care. Specifically, we found that MSM social isolation may be defined, in part, by the barriers to sexuality and HIV dual disclosures to parents who provide key support in healthcare access for this age group (4,32). Therefore, in order to bring MSM along with us in the global HIV strategy to get to the 95-95-95 goals by 2030, we need clinics and families with the skills to engage MSM in HIV treatment, and MSM-developed role-plays may be preferred and effective intervention components to facilitate this engagement (33,34).

Limitations

This study was conducted with a small group of HIV-positive MSM in a rural setting that may not be generalizable to all MSM experiences in South Africa nor other African settings. Yet, it is essential to reiterate that many African countries do not provide legal protections and human rights to sexual and gender minorities, which may limit the ability to organize groups to use participatory theatre to improve HIV outcomes (35).

Conclusion

Participants had a cathartic experience in the process of creating and performing role-plays about their experiences in HIV care. In this process, they developed self-advocacy points for them and other sexual and gender minorities as a means to claim their local clinics as their own and receive supportive HIV care rather than after developing AIDS-related illness. Our study suggests that role-play is a method to be further examined as a potential intervention component to build community among HIV-positive MSM, and then assess its impact on linkage to care in high HIV prevalence settings like Mpumalanga, South Africa.

Declaration of conflicting interests

The authors have no conflicts of interest to declare.

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Supplemental material

Supplemental material for this article is available online.

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